DR Care Solutions

Danielle Robertson Consulting Pty Ltd trading as DR Care Solutions

ABN 44 605 748 392 ACN 605 748 392
3408/1 Sergeants Lane, St Leonards NSW 2065

Life Plan™

This Life Plan™ is completed for all care needs assessments undertaken by a consultant from DR Care Solutions. It's used as a guideline as to the care options available for a specific client(s). It is a central place to capture your preferences around future ageing related care, types of support and living arrangements.

CLIENTS NAME: **CHRIS & VIVIENNE 'Viv' EVANS**
DOB: **18.01.1941 (84 years of age) and 02.12.1941 (83 years of age)**
CLIENT ID: **DRCS 0527**

Issues covered:	**Background and relevant history;** Nicola Said reached out to Danielle from DR Care Solutions on 19th December 2024 to discuss her parents ageing care and living needs after a decline in Chris and Viv's physical, cognitive and mental health. We had further discussions and a brief zoom with Chris, his wife Viv, Nicola and her brother Andrew on Thursday 2nd January 2025 to determine next steps. Chris agreed for Danielle to visit them in the family home in Winston Hills on Tuesday 7th January 2025 to assess Chris and Viv's current situation and provide suggestions and recommendations for support and accommodation options, now and into the future. Nicola was referred to Danielle and DR Care Solutions by Paul Irwin, the son of a current DR Care Solutions' client. **About Chris:** Chris has had some recent significant health issues and decline in his ability to make decisions. In August and September 2024, he underwent one operation on his heart as one artery was 95% blocked and they put in a stent and a second operation was for an abdominal aneurism. Chris has been Viv's primary carer since her formal dementia (Alzheimer's) diagnosis in 2024. She had been displaying significant signs for a few years prior. Since Chris' heart and abdominal surgery, his demeanour and personality has altered. Based on our extensive experience working with clients facing similar health issues, we've observed that the emotional burden of ongoing medical appointments, uncertainty about the future, and the limitations imposed by a chronic condition can lead to significant anxiety about health, medical procedures, and potential complications.

- Physical health including mobility, continence,
- Cognitive health
- Sensory (sight, hearing, oral, feet)
- Activities of daily living
- Emotional & mental health
- Risk assessment of the home and modification recommendations
- Medications, allergies & vaccinations
- External assistance - including Allied Health
- Social background, networks and Relationships

● Likes & dislikes ● Cultural linguistic and Spiritual needs ● Estate Plan/ Advice and Life Ready Register ● Financial Plan/ Advice ● My Aged Care ● Care and support considerations	This anxiety is often compounded by depression resulting from a loss of control, feelings of isolation, lifestyle restrictions, and the stress of balancing medical needs with work, relationships, and daily responsibilities. Additionally, clients frequently experience frustration over physical limitations and the unpredictability of their symptoms. Chris has been advised that he's pre-diabetic and food has now become a massive issue for him. He's lost 10kg in weight. He saw a dietician when he was in hospital and has had an additional consultation with another dietician. Previous issues have included skin cancers which have been frozen off or surgically removed. He has had prostate problems with urgency of micturition. Chris is independently mobile and doesn't require any aids to walk. His memory is fine however all he can think about is how difficult his life is currently. Chris has been to see a psychiatrist and psychologist. They've prescribed some anti-anxiety medications which he hasn't commenced taking yet. He's overly worried about salt, sugar and carbohydrates to a point where he has become obsessive and compulsive around these issues. He has also become obsessive on safe temperatures of food, food preparation. He has a very restricted diet and continues to lose weight and is additionally starting to feel continuously fatigued. He has developed sleep issues due to having a 'dry mouth', panic attacks and issues with his blood sugar. He is also on blood pressure medications. Chris has all his own teeth; he wears glasses for reading and driving. He has lost a lot of confidence with driving and only manages to drive to the local shops now. He doesn't have hearing aids and doesn't require them at this stage. Regular hearing tests are recommended. Chris has seen a professional podiatrist. We suggest that both he and Viv see a podiatrist, not just for nail and foot health, yet to check balance for falls prevention. The GP should be able to arrange 5 free visits through the GP health plan. We haven't received the GP Health Summary for Chris, therefore can't confirm any other diagnosed medical issues. Chris wakes at 5.30am, exercises (using the Nordic Track Recumbent Bike) and reads. He showers in the morning and spends his day managing appointments and planning their food intake for the day. He doesn't do much else apart from supervise Viv and make sure she's ok. Chris heads to bed around 9pm.

About Viv: Viv is physically very well. She enjoys walking and remaining active. She doesn't use any mobility aids and doesn't need them. She has arthritis and low blood pressure. Around 8 years ago, she had a breast cancer diagnosis and a lumpectomy with sixteen treatments of radiation.

Viv saw a Geriatrician, Dr Gary Cheuk from Blacktown in July / August 2024 who has diagnosed Alzheimer's Disease. Her dementia is progressing. Both her short- and long-term memory is affected. Viv remains very positive and upbeat in her personality. We haven't received the GP Health Summary for Viv therefore can't confirm any other diagnosed medical issues. Chris may have a copy however; we haven't received it.

Viv remains independent in showering, dressing, grooming and toileting. She does have some light bladder leakage and wears pads.

Viv has all her own teeth (with a few crowns). She wears glasses for reading. She hasn't visited a professional podiatrist however really should. As we outlined above, she should be able to have 5 free visits through her GP Health Plan.

Viv has hearing aids however doesn't tend to wear them. We suggest they are regularly reviewed and checked. It's recommended that Viv wears them when there are others around to keep her engaged in the conversation.

Viv wakes around 6am and she makes breakfast (with supervision) for them. She heads to bed around 9pm. Much of her day is spent on knitting and reading as well.

Risk assessment of the home and modification recommendations: The family home has been extremely well kept. Any rugs on the floors should be removed to avoid trip hazards. Sensor lights in the bathrooms and along hallways is recommended from the bedroom to the bathroom areas. If Chris and Viv decide to remain at home, we suggest an assessment by an Occupational Therapist is undertaken to ensure that the home remains safe and risk free for them to remain living there.

The home is large with four bedrooms and is situated over two storeys. The bedrooms are upstairs. Falls need to be prevented. There is a swimming pool and well-maintained gardens.

Medications, Allergies and vaccinations: **Chris and Viv** are fully vaccinated. We haven't received their GP Health Summaries so we can't confirm their medications however, Chris advised he is on statins and blood thinners. He bruises easily and bleeds profusely when he cuts himself.

Chris is allergic to penicillin and possibly another medication. He avoids anything with high sugar or salt content.

Viv isn't allergic to anything that she's aware of. All their medications are in their original boxes.

We strongly suggest that Chris and Viv arrange to have their medications and supplements placed into a Webster-Pak® arranged by his pharmacy on a fortnightly or monthly basis. This assists clients to avoid over or under medicating which often happens to Senior Australians. It also assists if a person needs to go into hospital or care, that all the medications are in the one spot and can be collected quickly.

We have a preferred partnership with Webstercare®. You can find a Pharmacy location where Webster-Paks® are supplied. Find a Webster-Pak® Pharmacy Near You | Webstercare

External assistance: Chris and Viv's son, Andrew is maintaining the pool and gardens and helps with driving them to appointments and general home maintenance. Nicola is shopping, preparing meals and delivering them regularly from the Central Coast to Winston Hills. She's no longer doing this as often as she was. She's almost at the end of her tether managing this on top of managing her own family, work and home life.

Nicola organised some cleaning for them a few months ago however they decided they didn't want it. They have since changed their mind and Nicola has re-sourced a provider with availability. She is still waiting on a call back to lock it in. If need be, Danielle can arrange a professional cleaning company to come in on a regular basis.

Viv and Chris maintain the home very well. They rarely leave the home unless it's for appointments or to purchase groceries. The home is well organised, uncluttered and clean. The bathrooms need a deep clean on a regular basis.

Social background, networks and relationships; Chris was born in South Wales, Swansea in the United Kingdom. He has a sister who is almost 6 years younger than him. She lives in Croxley Green in the North of London. She is fairly well however Chris mentioned that she has Atrial Fibrillation. He also has a half-sister living in Tel Aviv. His mother passed away at 92 years of age and his father at 62 years. His father was an ex-soldier living with PTSD (Post Traumatic Stress Disorder).

As a young man, Chris loved running and was very good at the half mile and full mile. He also played soccer and really enjoyed it. Chris attended primary and high school and then went on to Hatfield Polytechnic College to study engineering. He became an apprentice engineer.

	Chris was an engineer in the Defence Industry for 24+ years. As part of his employment, he used to travel extensively to the USA, Germany, Hong Kong, Europe, France, Japan and Canada. Chris and Viv met at a friend's party in Hadley, Shropshire in 1964. They both loved dancing and music from their era. Chris and Viv married in the 1960's. Both children were born in the UK. They had Nicola in 1967 and Andrew in 1970. They came to Australia in 1972 (53 years ago) and have remained in the same house in Winston Hills since that time. Andrew lives in Glenhaven and Nicola lives in Macmasters Beach (on the Central Coast). Nicola has two children, Elia (28 years) and Will (26 years). Elia has a new baby, Kaia (5 months old). Andrew has Tyler (16 years) and Connor (14 years). Chris retired from paid employment at the age of 70 (13 years ago). He enjoyed playing Golf and Bridge in his retirement. **Viv** was born in Hadley in the United Kingdom. Viv has two younger sisters; Jean who has passed away from Parkinsons Disease & dementia and Katreen who lives in Swansea in the UK. She has mobility issues however, no cognitive decline is evident. Viv attended Primary and High School and went to secretarial college. She learnt shorthand and became a secretary to a legal adviser at 'Castrol'. She also worked in Schools in the administration area until she had children. As a young girl, she enjoyed playing hockey and netball. She's very artistic, creative and loved painting, pottery and folk-art. Viv was an amazing seamstress and enjoyed quilting, knitting and sewing. Viv used to enjoy baking and loved to dance as well. She has a wonderful sense of humour. Chris and Viv used to have a wide social circle of friends from Chris' golf and his bridge club. Viv had a sewing group she belonged to and really enjoyed the social side of that. Chris has withdrawn and become introverted over the last 5 to 6 months. Viv remains socially isolated. **Likes and dislikes**: **Chris** enjoys eating Salmon (although he's concerned about his mercury intake due to eating it so often), chicken and beef. He doesn't eat any potatoes or pasta. He advised that he can't have anything with sugar in it. He used to drink alcohol (beer and wine) 3 to 4 drinks a day, however he no longer drinks. He was a social smoker may years ago (when he was a teenager).

	Chris used to love playing bridge, golf, dancing, running and gardening. He used to train soccer teams for Winston Hills Soccer Club. He loved musical theatre, theatre and going to the Opera House with Viv. **Viv** enjoys all food and eats everything placed in front of her. She has developed quite a sweet tooth and enjoys sweets and desserts. Viv has never smoked. She enjoys drinking red and white wine. Both say they drink plenty of water. Viv remains artistic and creative. She loves knitting. She spends most of her day doing that. She was a good homemaker prior to her diagnosis and still assists to clean and tidy the home. She enjoys musical theatre, theatre and going to the Opera House with Chris however, they don't go out much now apart from going to the shops to purchase groceries. **Cultural, linguistic and spiritual needs;** Neither **Chris** or **Viv** are religious. Chris can speak reasonable French and Viv can speak conversational French. They have no other cultural or spiritual needs currently. **Personal alarms and monitoring:** Currently they don't have a personal alarm or pendant. Depending on the overall decision as to next steps will determine whether we recommend that they obtain an Apple Watch (Series 7 and above) or something similar with falls detection and GPS tracking. If they are relocating into an assisted living environment, they won't need a personal alarm nor pendant as care will be on site. **Estate planning;** We understand that there is Enduring Guardianship and Powers of attorney in place which have recently been reviewed. We haven't sighted these documents however, we believe that Viv is Chris' Guardian and Power of Attorney and vice versa for Viv, ie: Chris is her Guardian and Power of Attorney. Given that Viv has a dementia diagnosis, and that Chris has been struggling with his mental health issues, the alternates listed are their children, Nicola Said and Andrew Evans. More recently, Nicola and Andrew have leaned in to support their parents who are struggling with the decision-making process. The four main documents we recommend are a will; Powers of Attorney (for financial decisions should either lose capacity), Enduring Guardianship (for any health, care or accommodation decisions should either lose capacity) and an Advanced Care Directive. For more information on Estate Planning for families, check the Estate Planning piece of the puzzle here: Aged Care Puzzle | DR Care Solutions |

	Advanced Care Directive: Below we include a special DR Care Solutions link to Touchstone Life Care for building an Advanced Care Directive. You are welcome to use this or ask your GP for a paper-based document if it's preferred. This is usually stored in the client's GP Health Summary and hospital records should something occur, the medical profession adheres to the requests if the person is unable to communicate their wants, needs and wishes. Touchstone Life Care - A DR Care Solutions Partner **Life Ready Register:** For DR Care Solutions clients, we provide a copy of the Life Ready Register, a depository to keep all your affairs in one place. We suggest that you download this and start completing it. No matter what age, everyone can commence building this out and sharing it with their trusted network, their Powers of Attorney and Enduring Guardians as well as their Executor of their Will. We often find the adult children of our clients using this document. Life Ready Register **Funeral arrangements:** We suggest considering organising Funerals well before it's required. We work very closely with Funeral Directors (from the Family Business Sector) and would be more than happy to introduce you to businesses that cover your area if requested. Be mindful of payment plans when it comes to funerals, as often they cost more than a standard funeral. Preplanning eases your family's burden during an emotionally difficult time. If you have specific questions, feel free to ask. **Financial planning;** Chris and Viv are mostly self-funded retirees. They have a part pension from the Government, an allocated pension from their Australian Superannuation and a senior's card. This doesn't mean that they can't access Government Funded Services. Some services are not means tested (Commonwealth Home Support Program which is entry level access to services within My Aged Care) and others are means tested (Home Care Packages, Residential respite care and permanent residential care). For more information on how the Government Support works, please review this section on our website. Government Support | DR Care Solutions. Chris requested that we introduce him to an Aged Care Accredited Financial Adviser which we did on the day of the assessment (7th January 2025). Chris had an initial discussion with the adviser to understand what's involved should he and Viv relocate into a retirement community or assisted living (residential aged care) as they are funded and structured very differently.

	My Aged Care: Nicola has registered both Chris and Viv with My Aged Care. She was advised that they would be assessed by February 2025 however she needed to chase them again. She has since advised that they've received Commonwealth Home Support Program referral codes for services from Western Sydney Local Health District assessor, Melanie See on 18th February 2024 for the following services: **Chris:** domestic Assistance; other food services and meals. **Viv**: social support individual, meals, flexible respite and other food services. **Please note** that these referral codes **are not for a Home Care Package** and that the family will need to search providers who offer these services on My Aged Care. There are long waitlists and often their waitlists are closed. The amount of support is minimal with different providers for each code coming through. Melanie has provided a phone number of 02 8890 3333 should urgent residential aged care be required. The codes have not been provided herein so we assume it would be for a short-term period rather than residential aged care respite with a view to permanent care. **Care and Support considerations:** Chris is very unsure about next steps for him and Viv. He is questioning whether having support at home is what he wants. He was clear in stating that something must change, ie: having help at home with the shopping and meal preparation. Nicola is only coming down once a week now. When she does come down from the Central Coast, she brings food. Andrew and Nicola, share the load and take them to appointments. Chris is challenging for them to manage currently. Nicola is finding it extremely difficult and won't be able to assist doing this long term. Nicola has been arranging meal deliveries every week which are mostly diabetic type meals to fit into Chris' rigid food regime but he's not overly loving them. This whole situation is affecting Nicola's own life and family. Additionally, Chris is eating the meals she's preparing sometimes yet does throw out a lot of food as he gets confused with spoilage, temperatures. There is a huge amount of wastage. Andrew is maintaining the pool and gardens; however, it is affecting both his and Nicola's lives and becoming increasingly frustrating them because Chris is resistant to change and moving forward. Chris articulated that he wants to find somewhere else to live however, he felt so overwhelmed.at the thought of it. He is unsure of how he can sort the current home and can't see the wood through the trees. Nicola advised this is very out of character of her father. From our 38 years in the homecare and aged care industry, we assess that Chris and Viv have two options available to them.

	OPTION A: stay in their current familial home and bring in additional care and support using any Government Supplements or Home Care Package funds in addition to privately funded (out of pocket) fee for service support. Chris advised he felt this was only an interim solution as he is 'over' living where he is and having all the responsibility for the home, gardens and caring for Viv. Our suggestion would be to bring in a professional carer(s) **two days a week for between 6 and 8 hours** to shop and prepare fresh meals, provide companionship for Viv and undertake some of the domestic duties including laundry and bedlinen changes. Additionally, provide them (or just Viv) with transport for outings and activities of interest (theatre, parks, exhibition, galleries etc) to enhance their quality of life. **OPTION B**: Relocate into a supported living environment, not a retirement community or independent living. This option comes with some sacrifices. Chris was clear he wants a **two-bedroom apartment** however we explained to Chris that in residential aged care, this is a rare occurrence. Some homes have larger suites yet, they won't have access to kitchen or cooking facilities. Some allow a bar fridge in the room. There are some options on the Central Coast where there are apartments with minimal support on site however this will mean possibly two moves in a short space of time particularly if Viv's dementia progresses quickly. We explained how retirement communities operate with no Government Funding provided and to consider deferred management fees and ongoing monthly costs. **Most retirement communities** don't have care or support on site unless you bring it in yourself (and pay for it) therefore you are no better off by moving. We suggest that clients over the age of 80 with declining health, avoid moving into a retirement community as it likely means two moves. Residential aged care is heavily funded by the Government. Residents have 24-hour care and support on site if / when they need it. Currently, with Chris' declining mental health and Viv's diagnosis of dementia, we believe this will be very good option with all their meals prepared, medications managed, laundry and cleaning undertaken, activities of interest and hobbies to undertake without the worry of maintaining a home, garden and swimming pool. Chris and Viv should obtain professional legal and financial advice around funding these options and understanding the contracts (particularly with retirement communities).

	Chris advised that if he were relocating into a different environment, he needs a large enough place for his computer (which is a tower computer), his own broadband internet (no Wi-Fi) and a place to keep his car. He wants a place with two bedrooms and preferably two bathrooms. He is fine to relocate to the Central Coast region of NSW closer to Nicola.
Goals (what does the client wish to achieve): ● Options ● Preferences ● Ideal Outcome ● Carer Profile	As above, Chris and Viv are unsure what the next steps are. Chris is unable to decide on this. He may need to chat this through with his psychiatrist, psychologist or GP. Nicola and Andrew have also attempted to support Chris in the decision-making process. If the wish is to **remain in the current home** with quality care and support in place, it is possible. We explained that getting a quality home care provider (with a Case Manager overseeing their care and support) having a supportive team around them to ramp up hours and days, 'as needed' is possibly what can occur. This may need to be partially funded by the Government and partially funded by Chris and Viv. As an **alternate option**, Chris and Viv could move into an assisted living environment or residential aged care for a period of respite (usually a minimum of three weeks) with a view to permanent care. If the decision is to remain at home, we've developed a carer profile for them to consider: **Carer Profile:** We envisage strong, caring, energetic, empathetic and highly skilled carers (male or female) be employed to support Chris and Viv to remain at home. Intelligent, well-travelled and educated carers, who can converse with them would be ideal. A care team could be gradually built up over time. A team of 2 carers (a mix of male and female carers) would be a great support to Chris and Viv, as needed' physically, emotionally, and intellectually. Carers from the local area would be preferred and with a clean driver's license and well-maintained vehicle. If the decision is to remain at home, they **will require a good cook** with knowledge of nutrition. The person will need to enjoy cooking and be able to use an air fryer. Chris wants someone with excellent food handling skills, hygiene and someone who knows how to prepare food for someone who is pre-diabetic. The main sticking point for Chris is the meals. Viv will eat anything. They must be low carb, low sugar and low salt. Chris wants everything to be high in meat protein. Nicola has been shopping and preparing the following meals to deliver to her parents:

	• Beef and Vegetable Pie (not too much pastry) • Beef and vegetable casserole • Lamb Roast with vegetables • Chilli Con Carne - (preferably not too many kidney beans or none and not spicy) • Cottage Pie (not too much potato) • Chicken and Vegetable Stir Fry with Brown Rice • Chicken and Vegetable Pie (not too much pastry) • Chicken Roast with Vegetables • Mild Curry Chicken or Beef with vegetables • Steak and Vegetables or Salad • Fish with vegetables or salad Chris felt he needed to purchase a back up microwave in case his breaks. We suggest he only considers doing that if he needs to. A carer to spend some time with Viv (as a companion) would be excellent. The selected carer could encourage Viv in the kitchen with them while preparing the food and have the ability to 'chat' with her. This would allow Chris to have a break. Homemaking skills would be advantageous to maintain the home including changing the bedlinen on both beds (they currently sleep in separate rooms as they both snore) therefore weekly bedlinen changes will be required, undertaking the washing and laundry, cleaning and drying the bathroom after showering but have the necessary 'carer' skills to eventually help Viv in the shower, dressing, undressing, toilet, bed, transfers 'as needed'. Assistance with shopping, carrying heavy groceries and cooking (as a high priority) is required. Carers who have clinical knowledge of someone living with Alzheimer's Disease, cognitive decline or mental health issues, who can identify any changes in Viv (and or Chris') condition would be helpful to relay back to their Case Manager (Generally a Registered Nurse). We encourage the carers to give Chris and Viv the space they need when they need it.
Actions required: (for the client to achieve these goals)	If the joint decision is to engage additional support at home, then the immediate focus is to get this support in place as quickly as possible 1. **Consider arranging immediate in-home care and support**. We can obtain quotes from providers. We will need estimated days and times that Chris and Viv are considering initially. We would suggest that at least **one or two days a week will be required initially for up to 6 to 8 hours** to ensure all the tasks are undertaken in one go.

	2. Request ACAT assessments (from My Aged Care). As part of ACAT assessments, please obtain additional referral codes for **a Home Care Package** *(If they decide to remain at home)* **as well as Residential Care (respite and permanent)**. We understand this has been arranged and booked in for early March 2025. 3. **Annually review existing Estate Planning Documents** including wills. Establish back up Powers of Attorneys, Enduring Guardians and Executors. 4. **Development of an Advanced Care Directive with** specific emphasis on healthcare and accommodation decisions. 5. **Complete the DR Care Solutions, Life Ready Register** and provide the completed document to their Next of Kin and Executors. Information provided in this life plan™. 6. Consider setting up a **prepaid funeral** in event of one or both passing. Please advise Danielle for an introduction to one of our Strategic Partners (Family Business) if interested. 7. The use of **personal alarms or monitoring devices** for emergencies such as falls and GPS Tracking (if they are remaining at home). 8. Chris and Viv to have **regular hearing tests** and obtain and / or service hearing aids if deemed necessary. 9. Chris and Viv to have their **eyes tested** and obtain new glasses if deemed necessary. 10. Both Chris and Viv to visit **a professional Podiatrist** to maintain foot health, nail health and aid balance (Check with GP as may be able to obtain five free visits per annum via the GP Health Plan).
Planning ahead and Implementation:	**Planning ahead:** Should something happen to **Viv**, Chris (in conjunction with his children) would need arrange for additional domestic support in the home to enable him to remain at home, long term (either fee for service or Government Funded or a combination of both). If this scenario occurred, Chris is likely to require more social interaction and time in the community to enhance his quality of life. A companion carer could be engaged to support Chris a few hours (minimum 3 hours), several times a week on top of the 'cooking' support.

	We can assist with sourcing a suitable carer for him through a reputable home care provider. Chris may consider staying where he is (at home) until the need to relocate into care or ramp up care and support in the home until end of life if that's his wish. Should something happen to **Chris,** the family (ie: Nicola and Andrew) would need to arrange immediate domestic support (including cleaning, shopping, cooking), companionship and care for Viv. Alternatively, we (at DR Care Solutions) could look to relocate Viv quickly and professionally into an assisted / supported living environment (Residential Aged Care) near Nicola on the Central Coast to ensure she is safe and well cared for and has social connection, particularly if Viv's physical condition deteriorates significantly (with double incontinence developing or continual falls or the escalation of her dementia). We recommend that anyone over the age of 80 years only has ONE move from their own home as it's stressful at the best of times, let alone being over 80. It can dramatically increase the progression of any disease if they need to relocate more than once (ie: from home, into a retirement community into residential aged care). **Implementation:** When the decision is made to implement this plan, we suggest that DR Care Solutions is engaged to select and introduce a suitable approved, pre-vetted Home Care Provider to support Chris and Viv to remain at home. We will introduce up to three pre-vetted quality home care providers (approved by the Government). One provider may have the perfect carers and support for them and therefore only one provider will be introduced. We need to ensure both their care needs are being met including meal preparation, occupation, physical activity, community access and socialisation. Consistency of carers is key to ensure both feel safe and comfortable. A team of at least **two skilled carers** should be able to cover the initial days needed. We always like to have at least two carers in a roster to ensure there is always someone who knows the client's routine and where things are kept, particularly if one carer is unwell or unavailable. A Case Manager is generally allocated to the family from the Home Care Provider, and they will visit them regularly to ensure there is complete oversight with the team of carers and ensure that they have the appropriate support at that time. A Case Manager is generally a Registered Nurse who can clinically quickly pick up any issues in relation to their condition, behaviour or clinical care needs.

	The rates being charged by providers 'fee for service' **range from $76 to $88 per hour** Monday to Friday for fully insured and employed carers (Minimum charge is generally 3 hours per visit). Weekends and public holidays incur surcharges. Review of the hours and their situation should be undertaken monthly by the Case Manager and relayed back to the family to ensure the right amount of care and support services are taking place for them at any given time. Government funding can be added to the mix of funding once it comes through. Home Care packages through the Government are means tested. Some providers offer a LIVE-IN service (24 hours a day) with one carer per day. The client needs to be assessed (by the provider) as to whether they need support overnight or just need someone on site in case of emergency. We can discuss this option, if or when it is required. Other providers only offer coverage of the 24 hours in 8 hour or 12 hour rotating shifts. We will guide and support you through this process to get the right outcome for Chris and Viv. As outlined above, should either Chris or Viv's condition deteriorate significantly to a point where they can't be cared for at home safely, we can be **re-engaged** to find the right Aged Care Home for respite with a view to permanent care. We run a comparative platform report which brings up the top 10 Aged Care homes in a certain area. We work closely with the family to find the right home for the client (or both if needed) and tour up to four or five in a particular location. It's not just locating a bed for a client, it's their next home. We personally ensure that the selected home is appropriate for the client's needs, wants and wishes (currently and into the future) within their financial capacity to afford the home and that they are clinically accepted by the home. It's not just the family determining the home We undertake all the groundwork, pre-vetting and due diligence of the aged care homes and remove the pressure and stress out of finding the right home at an already emotionally charged time. We walk alongside you for as long as the client needs care or is in care.
Resources:	Explore Aged Care Puzzle, a resource designed to guide you through every step of the aged care journey. With our step-by-step puzzle solution, you can confidently make informed choices at your own pace. Just pick your first Aged Care Puzzle piece, and you're on your way! Aged Care Puzzle | DR Care Solutions | DR Care Solutions

Who will be involved: (Guardian, solicitor, financial planner/advisers, family members)	• Chris and Viv Evans, Nicola Said and Andrew Evans • Danielle Robertson from DR Care Solutions • Financial adviser / Accountant • Estate Lawyer (review of all estate planning documents) • GP, Geriatrician, Psychiatrist, Psychologist and any other medical specialists
Review of Life Plan and the date the Family need to determine next steps:	Friday, 7th March 2025

Signature (Consultant):  Date: Monday 3rd March 2025

This document is issued by Danielle Robertson Consulting Pty Limited ABN 44 605 748 392 trading as DR Care Solutions. This document contains proprietary information of DR Care Solutions and is intended for the exclusive use of the parties to whom it is provided by DR Care Solutions. Its content may not be modified, sold or otherwise provided, in whole or in part, to any other person or entity without DR Care Solutions prior written permission. © 2025 Danielle Robertson Consulting Pty Limited. All Rights Reserved